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Drug-Resistant Bacterial Infection Prevention Training

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Disclosures



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Learning Objectives



- To describe the key steps in designing entrustable professional activities (EPAs).
- To identify approaches for piloting and integrating EPAs within training programs and certification systems.
- To recognize barriers to EPA implementation and apply strategies to promote equitable, unbiased assessment.

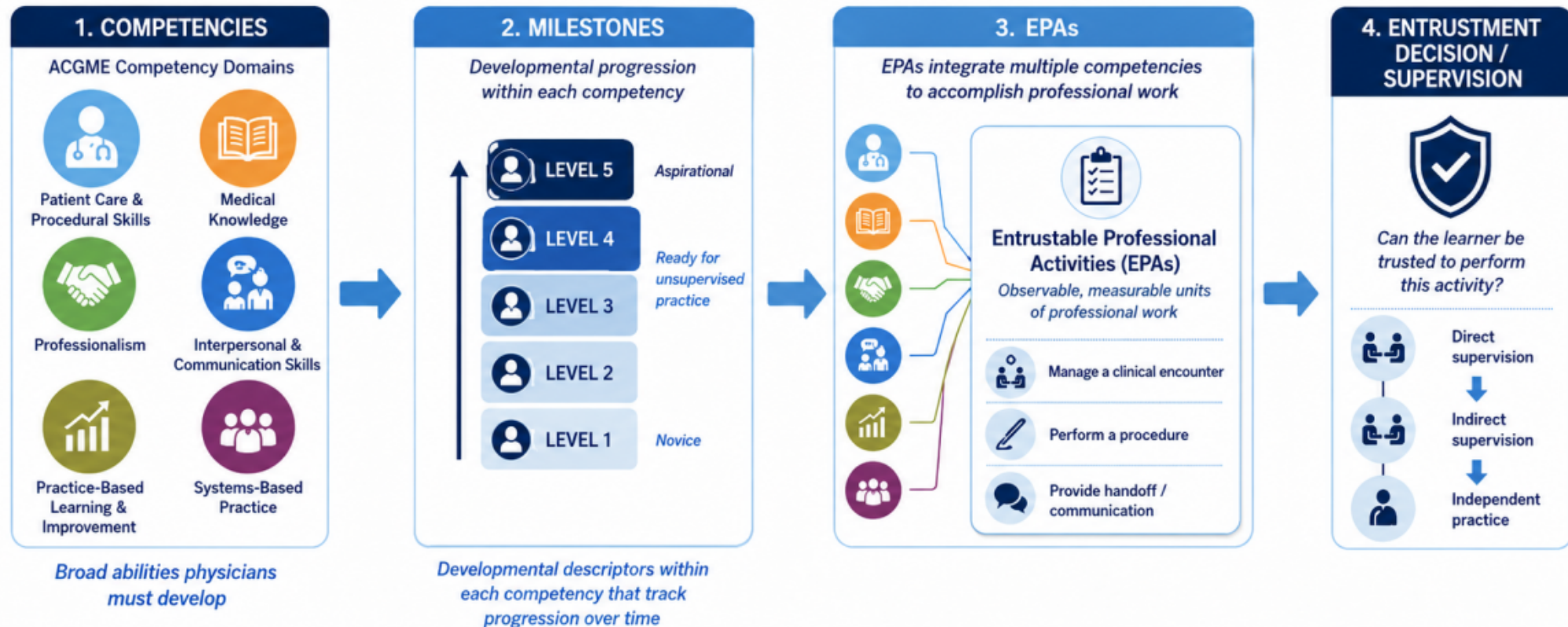


Background

Definition



How Competencies, Milestones, and EPAs Relate



Competencies describe abilities. Milestones describe development. EPAs describe the work a learner can be trusted to do.



GET CERTIFIED

EPAs for General Surgery

General surgery EPAs launched for residency programs in July 2023, and is now in the implementation phase for the other ABS specialties.

[EPAs for General Surgery - American Board of Surgery](#)

COMMENTARY: EXECUTIVE SUMMARY

A Framework for Building Surgical Competence in the Practice of Gynecology A Discussion From the 2024 American Gynecological & Obstetrical Society Annual Meeting

Alvarez, Ronald D. MD, MBA; Huh, Warner K. MD, MSHA; Connolly, AnnaMarie MD; Young, Amy E. MD; Alston, Meredith J. MD; Advincula, Arnold P. MD; Dalrymple, John L. MD; Johnson, Donna D. MD; Goff, Barbara A. MD; Cedars, Marcelle I. MD; Coutifaris, Christos MD, PhD

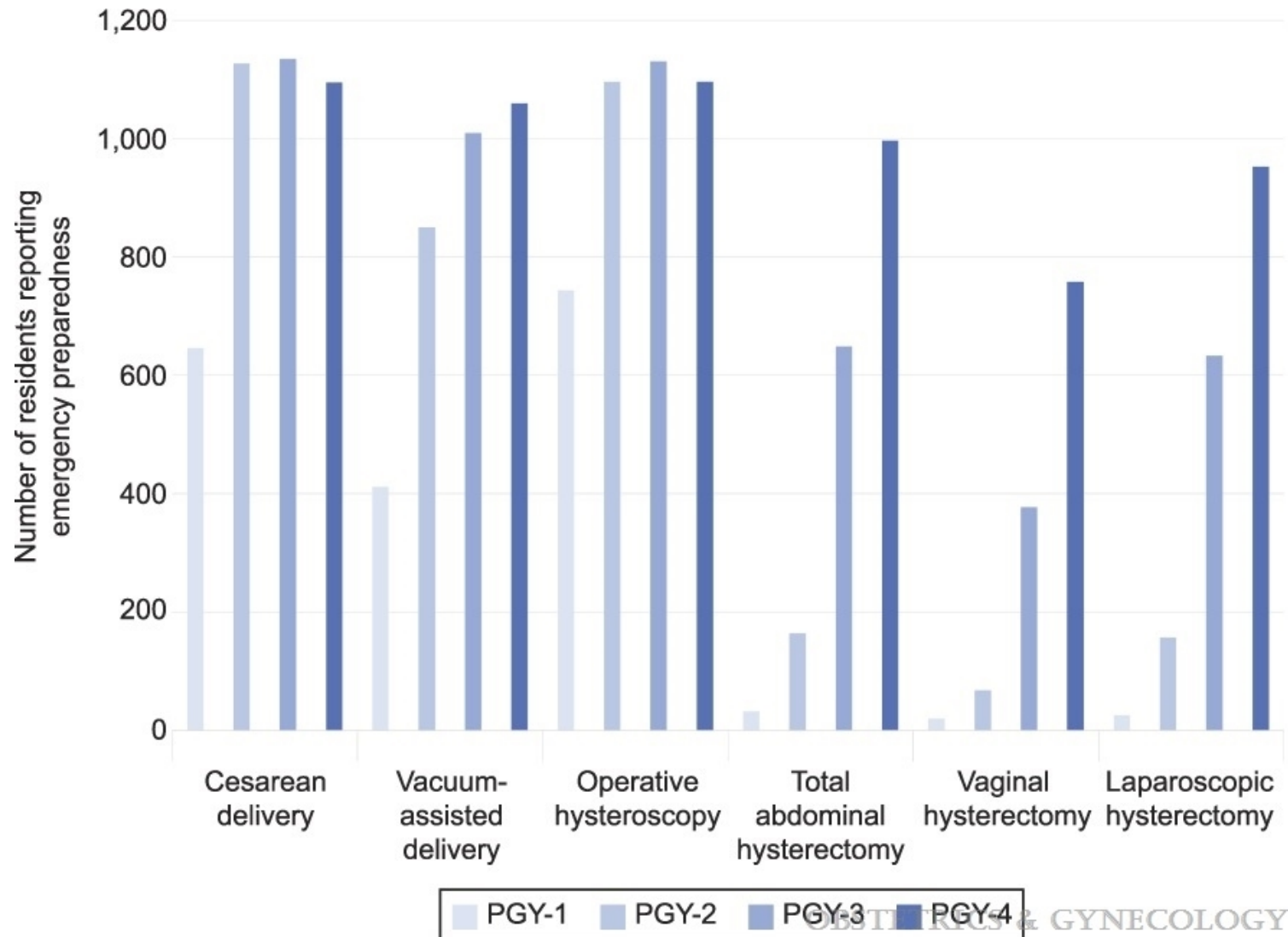
[Author Information](#) ✓

Obstetrics & Gynecology 146(1):p 27-33, July 2025. | DOI: 10.1097/AOG.0000000000005925



Case Study

Training in OVD



Resident confidence in performing obstetric and gynecologic surgical procedures in the event of an emergency. PGY, postgraduate year.
Banks. Obstetrics and Gynecology Residents' Surgical Confidence. Obstet Gynecol 2020

Project Aims



To develop a consensus-defined OVD-specific EPA that delineates the core functions expected of OB/GYN trainees



To evaluate the feasibility of implementing an OVD-specific EPA and corresponding supervisory-scale entrustment rubric in OB/GYN clinical training settings



To assess the impact of OVD-specific EPA and entrustment rubric implementation on faculty entrustment decisions and on trainee confidence in performing OVD

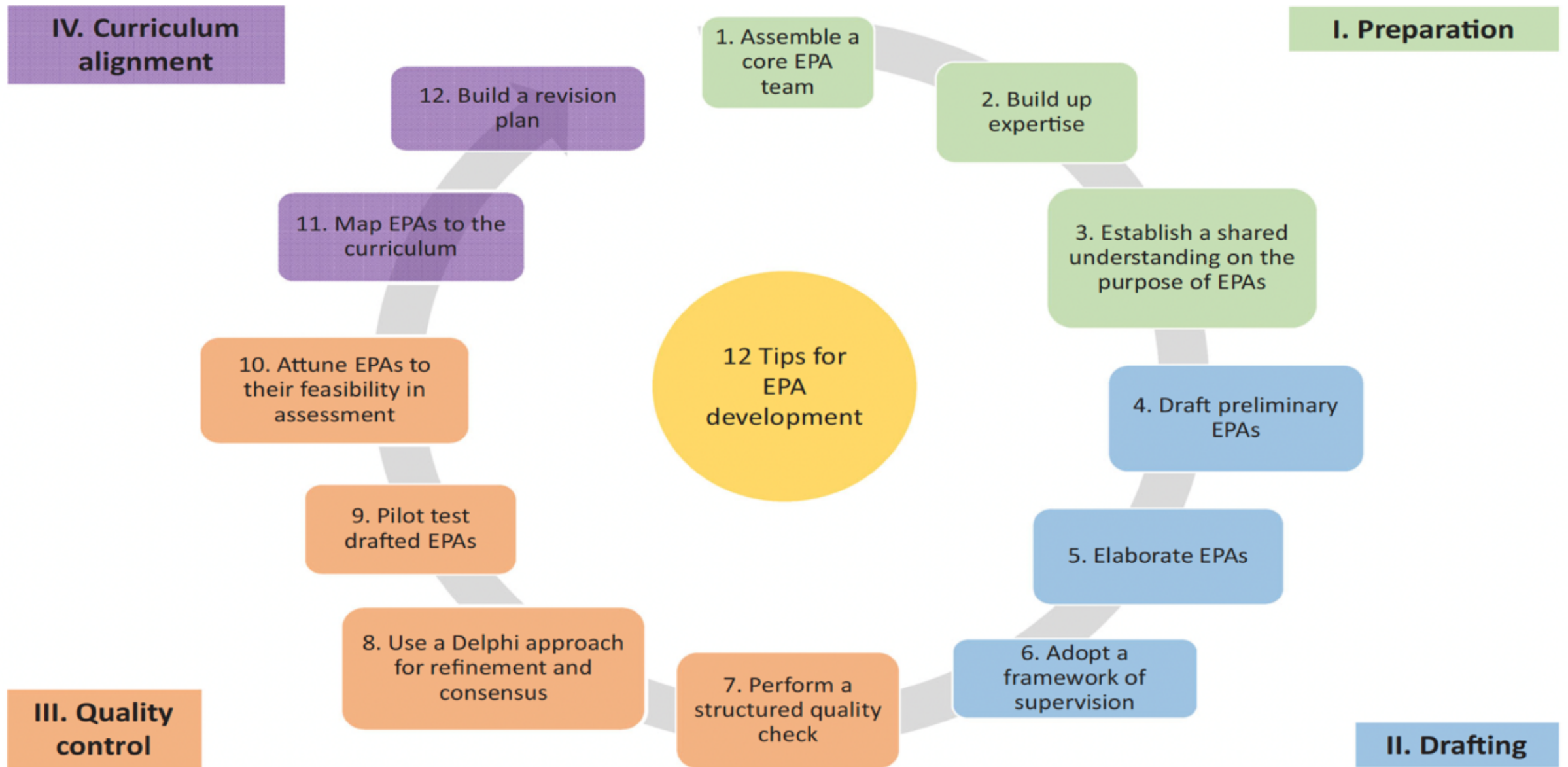
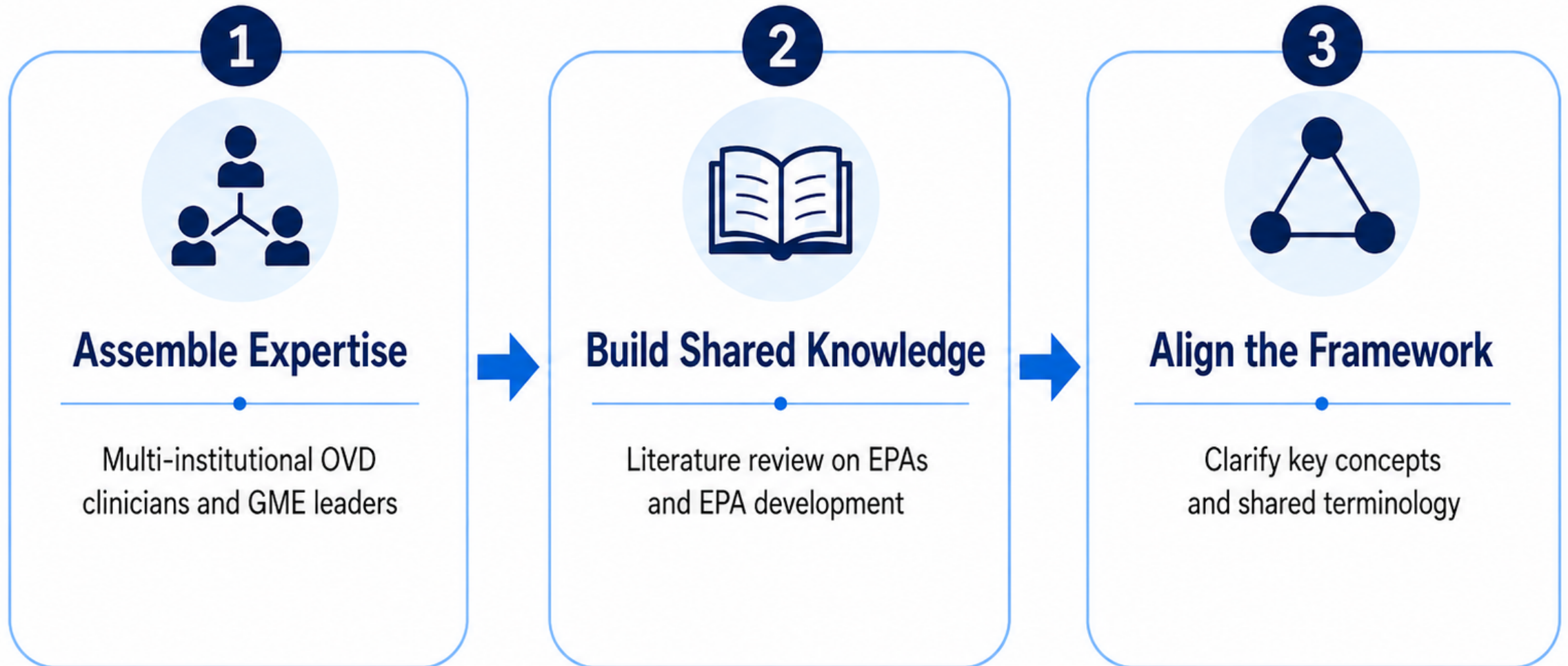


Figure 1. Flowchart with sequential and practical tips for Entrustable Professional Activities (EPA) development.

Hennus MP, Jarrett JB, Taylor DR, Ten Cate O. Twelve tips to develop entrustable professional activities. *Med Teach*. 2023;45(7):701-707. doi:10.1080/0142159X.2023.2197137

Steps 1-3: Preparation Phase



Steps 4-5: Draft and Elaborate Preliminary OVD-Specific EPA



Grounded in OVD literature + national EPA frameworks

Nominal Group Technique

Structured process:

1



Independent idea
generation > pre-meeting



2



Round-robin
sharing



3



Clarification &
discussion



4



Prioritization/ranking
> post-meeting



Begin EPA elaboration:
scope, limitations, risks



Result: Preliminary
OVD-specific EPA

Steps 6-7: Adopt a framework of supervision and structured quality check

Adapt supervisory scale from Chen et al.

Progression reflects real clinical autonomy:

- Proactive supervision
- Reactive/indirect supervision
- Unsupervised practice
- Ability to supervise others

EPA Quality Check (EQual Rubric):

- Clear, observable clinical task
- Integrates multiple competencies
- Appropriate scope and feasibility
- Defines risks & supervision expectations

Chen HC, van den Broek WE, ten Cate O. The case for use of entrustable professional activities in undergraduate medical education. *Acad Med.* 2015;90(4):431-436.

Taylor DR, Park YS, Egan R, et al. EQual, a Novel Rubric to Evaluate Entrustable Professional Activities for Quality and Structure. *Acad Med.* 2017;92(11S Association of American Medical Colleges Learn Serve Lead: Proceedings of the 56th Annual Research in Medical Education Sessions):S110-S117.



Step 8: Delphi Approach



Recruitment and survey distribution



- Recruitment emails sent to program directors and associate program directors at 160 institutions



- 53 participants initially agreed to participate, representing 45 programs



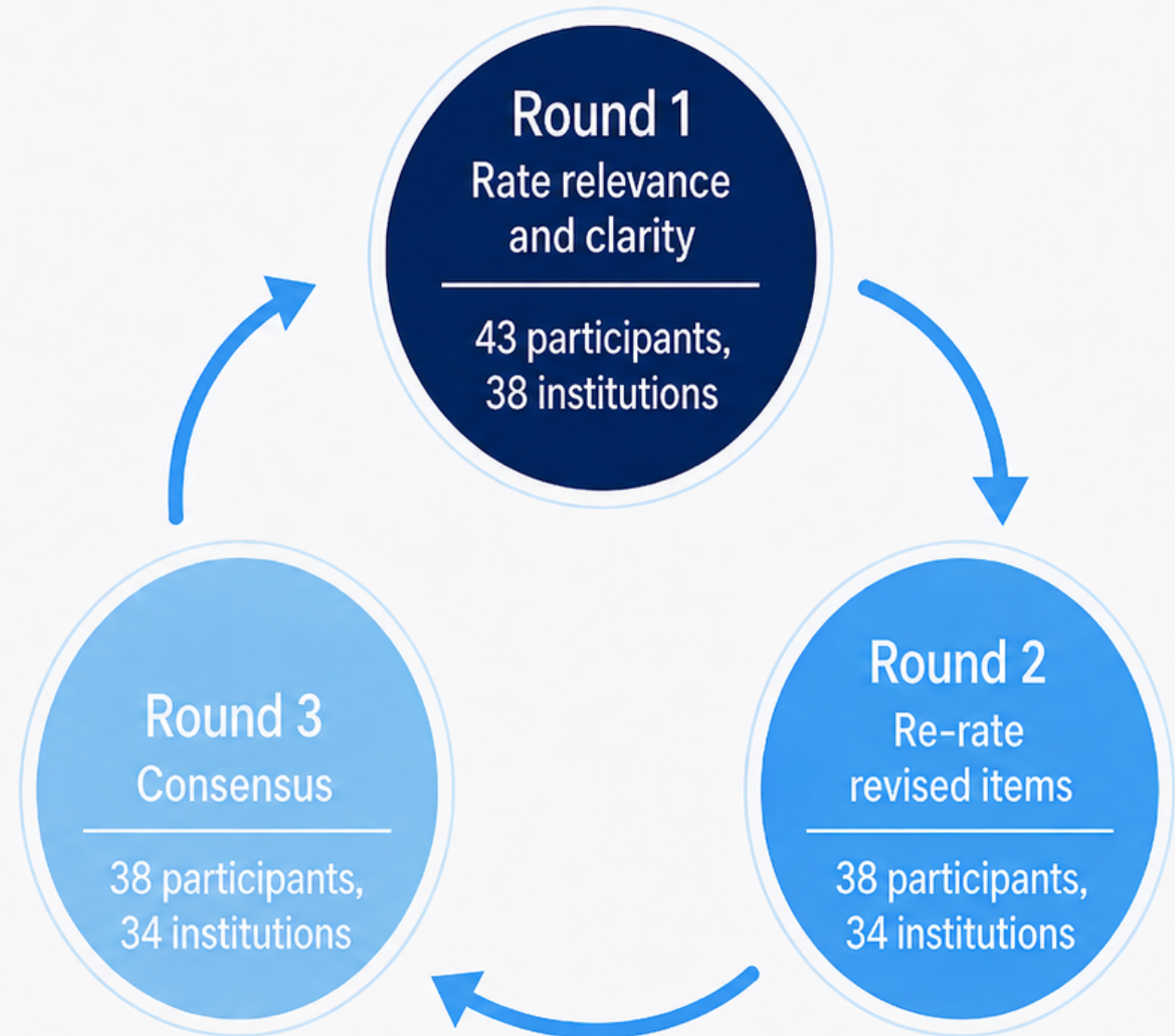
- Several reminder emails were sent for each round



- Participants received a \$50 gift card as an incentive for completing all three rounds



Three-Round Modified Delphi Process





Small group discussion



Equity check: At this stage of EPA development or implementation, where could subjectivity, unequal opportunity, or bias enter the process? What is one specific design or implementation strategy you could use to reduce that risk?

Group 1:
We are developing EPAs.

Group 2:
We are preparing to pilot or implement EPAs.

Group 3:
We are implementing EPAs and encountering challenges.

Group 4:
We are scaling EPAs to specialty-wide assessment and/or certification.



Group 1: We are developing EPAs.



What are the essential activities of an independently practicing physician in your specialty?

Identify 2-3 activities.

Does it represent a meaningful, observable unit of professional work rather than an isolated skill or competency?

For one possible EPA, define its scope.

Where does the activity begin and end?

What tasks or decisions are included?

What is considered out of scope or requiring greater supervision?

What must a trainee do to be entrusted with this activity?

Identify 4-6 essential functions.

Consider clinical assessment, decision-making, communication, procedures/management, teamwork/escalation, and follow-up.

How would you establish that this is the "right" EPA for your specialty?

Who needs to be involved in development and validation?

How would you build consensus while retaining clinical usefulness?

Group report out: Proposed EPA, 2–3 essential functions, and one important boundary or exclusion.

Group 2: We are preparing to pilot EPAs.



Where do the EPAs naturally occur in clinical workflow?

Who is best positioned to observe it?

When could an assessment be initiated with the least disruption to patient care?

What would a pilot look like?

Consider which learners, assessors, clinical settings, and program(s) should participate.

What would you want to learn about expanding before implementation?

How will you create a shared understanding of entrustment?

What assessor development, behavioral anchors, sample cases, calibration exercises, or learner orientation would help assessors use supervision levels consistently?

How will EPA data move from individual workplace-based assessment to decisions about progression?

Who reviews the data?

How many different observations, assessors, settings, or types of evidence are needed before making a higher-stakes entrustment decision?

Group report out: One workflow strategy, one faculty/learner development strategy, and one outcome you would measure during the pilot.

Group 3: We are implementing EPAs and encountering challenges.



Six months after implementation, assessment completion is much lower than expected. Why?

Consider learner engagement, assessor buy-in, integration into clinical workflow, assessment burden, technology, opportunities for observation, and competing priorities.

Which barrier would you address first and how?

Two assessors observe similar learner performance but consistently assign different supervision levels. What would you do?

How could behavioral anchors, calibration exercises, faculty development, assessment data, or group review improve assessor alignment without eliminating appropriate clinical judgement?

How will you know whether EPA implementation is actually improving assessment?

Identify 2-3 meaningful outcomes (e.g., assessment completion, quality of feedback, progression of supervision ratings, learner/assessor acceptability, time burden, or identification of struggling learners).

Group report out: Highest-priority implementation barrier, one intervention, and one measure that would tell you whether the intervention worked.

Group 4: We are scaling EPAs to specialty-wide assessment and/or certification.



What evidence should be required before EPA data contribute to a high-stakes decision such as graduation or board eligibility?

What would constitute a sufficiently rich body of evidence rather than relying on a single rating or arbitrary number of assessments?

What should be standardized nationally, and what should remain locally adaptable?

Consider EPA definitions, supervision scales, minimum evidence, assessor development, assessment platforms, and interpretation by competency committees.

How should differences in clinical exposure across programs be handled?

What happens when a learner has limited opportunity to be observed performing a particular EPA because of case volume, site structure, or subspecialty practice patterns?

What safeguards are needed before using EPA data for certification?

Consider validity evidence, assessor variation, data completeness, transparency to trainees, remediation, and processes for reviewing unexpected or discordant findings.

Group report out: One element that must standardized, one element that should remain flexible, and one guardrail you would require before using EPA data for certification.



Large group discussion

Large group discussion prompts



What challenges appeared across specialties despite differences in clinical practice?



What aspects of EPA development and implementation should be standardized across a specialty, and where is local adaptation necessary?



Where in the EPA process, from defining the activity to making an entrustment decision, is there the greatest risk for inequity or bias?



If you could take one strategy from another specialty represented in the room back to your own program or board, what would it be?

Key Takeaways



Advancing CBME Through Collaboration

Multispecialty collaboration across institutions, specialties, and training programs is essential to advance CBME.



Specialty-Specific EPAs Drive Trust and Readiness

EPAs translate the work we do into observable, measurable activities that support transparent entrustment decisions and learner readiness for independent practice.



From Pilots to Practice: Building the Future of Assessment

Piloting, integrating, and continuously refining EPAs will strengthen assessment, reduce bias, and ensure a competent, practice-ready workforce—advancing certification for the future.



OVD is our case study. The impact is broader.
Together, we are building a stronger CBME ecosystem.



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